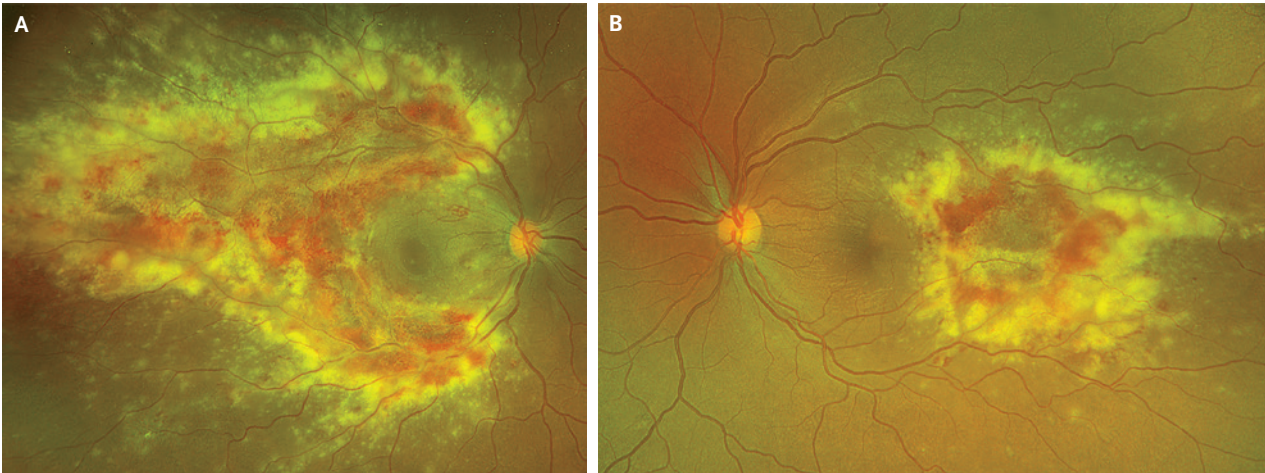


IMAGES IN CLINICAL MEDICINE

Stephanie V. Sherman, M.D., *Editor*

Cytomegalovirus Retinitis



Nicolas Kitic, M.D.
Bahram Bodaghi, M.D., Ph.D.

Assistance Publique–Hôpitaux de Paris–
Sorbonne Université
Paris, France
bahram.bodaghi@aphp.fr

A 34-YEAR-OLD WOMAN WITH HUMAN IMMUNODEFICIENCY VIRUS (HIV) infection presented to the ophthalmology emergency department with a 3-day history of a blind spot in her right eye. Seven months earlier, she had received diagnoses of HIV infection and tuberculous lymphadenitis and had started receiving antiretroviral and antituberculosis therapies. Two months before presentation, treatment with prednisone had been initiated for tuberculosis-associated immune reconstitution inflammatory syndrome. One week before the current presentation, the HIV viral load was 57 copies per milliliter (reference value, <20) and the CD4 cell count was 81 per cubic millimeter (reference range, 500 to 1500). On eye examination, visual acuity was 20/50 in the right eye and 20/20 in the left eye. Funduscopy of the right eye revealed fulminant retinitis with dense areas of retinal necrosis and hemorrhage (Panel A). The left eye, which had been asymptomatic, had similar changes (Panel B). To confirm a viral retinitis, anterior chamber paracentesis was performed, and a high viral load of cytomegalovirus (CMV) was found on quantitative polymerase-chain-reaction assay. A diagnosis of CMV retinitis was confirmed. Treatment with intravenous foscarnet and intravitreal ganciclovir was initiated. One month after the completion of treatment, retinal detachment occurred in both eyes. Four months after surgical repair of the retinas, the patient's visual impairment had stabilized.

DOI: 10.1056/NEJMicm2405496

Copyright © 2025 Massachusetts Medical Society.